Consolidated Continuity of Care Document

Patient	
Date of birth	
Sex	Male
Race	
Ethnicity	
Contact info	
Patient IDs	
Document Id	
Document Created:	July 7, 2022, 10:49+0000

Author	CCDA Generator v1.0
Contact info	
Document maintained by	HASA
Contact info	

Problems

Name	Dates	Details	Sending Facility
HEART FAILURE, UNSPECIFIED	2021-02-25 15:40	Type: ICD-10; Value: I50.9; Descr: HEART FAILURE, UNSPECIFIED	Methodist Texsan Hospital
UNSPECIFIED ATRIAL FIBRILLATION	2021-02-25 15:40	Type: ICD-10; Value: I48.91; Descr: UNSPECIFIED ATRIAL FIBRILLATION	Methodist Texsan Hospital
HYP HRT CHR KDNY DIS W HRT FAIL AND STG 14/UNSP CHR KDNY	2021-02-25 16:53	Type: ICD-10; Value: I13.0; Descr: HYP HRT CHR KDNY DIS W HRT FAIL AND STG 1-4/UNSP CHR KDNY	Methodist Texsan Hospital
HYP HRT CHR KDNY DIS W HRT FAIL AND STG 14/UNSP	2021-02-25 16:53	Type: ICD-10; Value: I13.0; Descr: HYP HRT CHR KDNY DIS W HRT FAIL AND STG 1-4/UNSP	Methodist Texsan Hospital
ACUTE ON CHRONIC DIASTOLIC (CONGESTIVE) HEART FAIL	2021-02-25 16:53	Type: ICD-10; Value: I50.33; Descr: ACUTE ON CHRONIC DIASTOLIC (CONGESTIVE) HEART FAIL	Methodist Texsan Hospital
ACUTE KIDNEY FAILURE, UNSPECIFIED	2021-02-25 16:53	Type: ICD-10; Value: N17.9; Descr: ACUTE KIDNEY FAILURE, UNSPECIFIED	Methodist Texsan Hospital
TYPE 2 DIABETES MELLITUS W DIABETIC CHRONIC KIDNEY	2021-02-25 16:53	Type: ICD-10; Value: E11.22; Descr: TYPE 2 DIABETES MELLITUS W DIABETIC CHRONIC KIDNEY	Methodist Texsan Hospital
HYPERLIPIDEMIA, UNSPECIFIED	2021-02-25 16:53	Type: ICD-10; Value: E78.5; Descr: HYPERLIPIDEMIA, UNSPECIFIED	Methodist Texsan Hospital
ATHSCL HEART DISEASE OF NATIVE CORONARY ARTERY W/O	2021-02-25 16:53	Type: ICD-10; Value: I25.10; Descr: ATHSCL HEART DISEASE OF NATIVE CORONARY ARTERY W/O	Methodist Texsan Hospital
CHRONIC KIDNEY DISEASE, STAGE 2 (MILD)	2021-02-25 16:53	Type: ICD-10; Value: N18.2; Descr: CHRONIC KIDNEY DISEASE, STAGE 2 (MILD)	Methodist Texsan Hospital
LONG TERM (CURRENT) USE OF INSULIN	2021-02-25 16:53	Type: ICD-10; Value: Z79.4; Descr: LONG TERM (CURRENT) USE OF INSULIN	Methodist Texsan Hospital
HYPERKALEMIA	2021-02-25 16:53	Type: ICD-10; Value: E87.5; Descr: HYPERKALEMIA	Methodist Texsan Hospital
LONG TERM (CURRENT) USE OF ASPIRIN	2021-02-25 16:53	Type: ICD-10; Value: Z79.82; Descr: LONG TERM (CURRENT) USE OF ASPIRIN	Methodist Texsan Hospital
ACUTE AND SUBACUTE HEPATIC FAILURE WITHOUT COMA	2021-02-25 16:53	Type: ICD-10; Value: K72.00; Descr: ACUTE AND SUBACUTE HEPATIC FAILURE WITHOUT COMA	Methodist Texsan Hospital

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CONTACT WITH AND (SUSPECTED) EXPOSURE TO COVID-19	2021-02-25 16:53	Type: ICD-10; Value: Z20.822; Descr: CONTACT WITH AND (SUSPECTED) EXPOSURE TO COVID-19	Methodist Texsan Hospital
ACUTE ON CHRONIC COMBINED SYSTOLIC AND DIASTOLIC H	2021-02-25 16:53	Type: ICD-10; Value: I50.43; Descr: ACUTE ON CHRONIC COMBINED SYSTOLIC AND DIASTOLIC H	Methodist Texsan Hospital
ACIDOSIS	2021-02-25 16:53	Type: ICD-10; Value: E87.2; Descr: ACIDOSIS	Methodist Texsan Hospital
ENCEPHALOPATHY, UNSPECIFIED	2021-02-25 16:53	Type: ICD-10; Value: G93.40; Descr: ENCEPHALOPATHY, UNSPECIFIED	Methodist Texsan Hospital
VENTRICULAR TACHYCARDIA	2021-02-25 16:53	Type: ICD-10; Value: I47.2; Descr: VENTRICULAR TACHYCARDIA	Methodist Texsan Hospital
BODY MASS INDEX [BMI]40.0-44.9, ADULT	2021-02-25 16:53	Type: ICD-10; Value: Z68.41; Descr: BODY MASS INDEX [BMI]40.0-44.9, ADULT	Methodist Texsan Hospital
OTHER SECONDARY THROMBOCYTOPENIA	2021-02-25 16:53	Type: ICD-10; Value: D69.59; Descr: OTHER SECONDARY THROMBOCYTOPENIA	Methodist Texsan Hospital
HYPOMAGNESEMIA	2021-02-25 16:53	Type: ICD-10; Value: E83.42; Descr: HYPOMAGNESEMIA	Methodist Texsan Hospital
HYPOKALEMIA	2021-02-25 16:53	Type: ICD-10; Value: E87.6; Descr: HYPOKALEMIA	Methodist Texsan Hospital
OTHER CHRONIC PAIN	2021-02-25 16:53	Type: ICD-10; Value: G89.29; Descr: OTHER CHRONIC PAIN	Methodist Texsan Hospital
ATHSCL HEART DISEASE OF NATIVE COR ART W OTH ANG P	2021-02-25 16:53	Type: ICD-10; Value: I25.118; Descr: ATHSCL HEART DISEASE OF NATIVE COR ART W OTH ANG P	Methodist Texsan Hospital
ANXIETY DISORDER, UNSPECIFIED	2021-02-25 16:53	Type: ICD-10; Value: F41.9; Descr: ANXIETY DISORDER, UNSPECIFIED	Methodist Texsan Hospital
NEPHROPATHY INDUCED BY OTH DRUG/MEDS/BIOL SUBST	2021-02-25 16:53	Type: ICD-10; Value: N14.1; Descr: NEPHROPATHY INDUCED BY OTH DRUG/MEDS/BIOL SUBST	Methodist Texsan Hospital
ADVERSE EFFECT OF DIAGNOSTIC AGENTS, INITIAL ENCOU	2021-02-25 16:53	Type: ICD-10; Value: T50.8X5A; Descr: ADVERSE EFFECT OF DIAGNOSTIC AGENTS, INITIAL ENCOU	Methodist Texsan Hospital
POISONING BY UNSP DRUG/MEDS/BIOL SUBST, ACCIDENTAL	2021-02-25 16:53	Type: ICD-10; Value: T50.901A; Descr: POISONING BY UNSP DRUG/MEDS/BIOL SUBST, ACCIDENTAL	Methodist Texsan Hospital
OBESITY, UNSPECIFIED	2021-02-25 16:53	Type: ICD-10; Value: E66.9; Descr: OBESITY, UNSPECIFIED	Methodist Texsan Hospital
DORSALGIA, UNSPECIFIED	2021-02-25 16:53	Type: ICD-10; Value: M54.9; Descr: DORSALGIA, UNSPECIFIED	Methodist Texsan Hospital
CARDIAC MURMUR, UNSPECIFIED	2021-02-25 16:53	Type: ICD-10; Value: R01.1; Descr: CARDIAC MURMUR, UNSPECIFIED	Methodist Texsan Hospital
ST ELEVATION (STEMI) MYOCARDIAL INFARCTION OF UNSP	2021-02-25 16:53	Type: ICD-10; Value: I21.3; Descr: ST ELEVATION (STEMI) MYOCARDIAL INFARCTION OF UNSP	Methodist Texsan Hospital
TOXIC ENCEPHALOPATHY	2021-02-25 16:53	Type: ICD-10; Value: G92; Descr: TOXIC ENCEPHALOPATHY	Methodist Texsan Hospital
ATRIAL PREMATURE DEPOLARIZATION	2021-02-25 16:53	Type: ICD-10; Value: I49.1; Descr: ATRIAL PREMATURE DEPOLARIZATION	Methodist Texsan Hospital
OTHER PROBLEMS RELATED TO LIFESTYLE	2021-02-25 16:53	Type: ICD-10; Value: Z72.89; Descr: OTHER PROBLEMS RELATED TO LIFESTYLE	Methodist Texsan Hospital
FAMILY HX OF ISCHEM HEART DIS AND OTH DIS OF THE C	2021-02-25 16:53	Type: ICD-10; Value: Z82.49; Descr: FAMILY HX OF ISCHEM HEART DIS AND OTH DIS OF THE C	Methodist Texsan Hospital
FAMILY HISTORY OF DIABETES MELLITUS	2021-02-25 16:53	Type: ICD-10; Value: Z83.3; Descr: FAMILY HISTORY OF DIABETES MELLITUS	Methodist Texsan Hospital
LONG TERM (CURRENT) USE OF OPIATE ANALGESIC	2021-02-25 16:53	Type: ICD-10; Value: Z79.891; Descr: LONG TERM (CURRENT) USE OF OPIATE ANALGESIC	Methodist Texsan Hospital
ALLERGY STATUS TO NARCOTIC AGENT	2021-02-25 16:53	Type: ICD-10; Value: Z88.5; Descr: ALLERGY STATUS TO NARCOTIC AGENT	Methodist Texsan Hospital
OTH PLACES AS THE PLACE OF OCCURRENCE OF THE EXTER	2021-02-25 16:53	Type: ICD-10; Value: Y92.89; Descr: OTH PLACES AS THE PLACE OF OCCURRENCE OF THE EXTER	Methodist Texsan Hospital
LONG TERM (CURRENT) USE OF ANTITHROMBOTICS/ANTIPLA	2021-02-25 16:53	Type: ICD-10; Value: Z79.02; Descr: LONG TERM (CURRENT) USE OF ANTITHROMBOTICS/ANTIPLA	Methodist Texsan Hospital

Social History

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Medications

Transcriptions

Universal Serivce Identifier - Text	Date	Paragraph
ECHO| ECHOCARDIOGRAM REPORT	202106-09 15:31	1310 MCCULLOUGH SAN ANTONIO, TEXAS 78212 PATIENT'S NAME: B UNIT NO: DOB: AGE: 67 SEX: M ACCOUNT NO: ATTENDING PHYS: Dr. Roger K Muse, MD PT TYPE: PRE REF ROOM NO: DATE OF ADMISSION: DATE OF DISCHARGE: Metropolitan Methodist Hospital 1310 McCullough Avenue San Antonio, Texas 78212 210-757-2200 Transthoracic Echocardiogram 2D Spectral Color Flow Doppler Name: WALLACE, LARRY MPI#: X1821

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Facility MR#: Admission Number:
Study Date: 06/09/2021 Study Time:
02:20 PM
Date Of Birth: Age:
67 year(s)
Height: 71
in. (180.3
cm)
Weight:
253.4 lbs.
(114.94 kg)
BSA: 2.33
m2Gender:
Male
Blood Pressure: 140 mmHg / 75 mmHgHeart Rate:
75 bpmRhythm: Normal sinus rhythm
Infection: Emergency: Contrast Allergy:

Procedure StaffReferring Physician: Roger Muse MD
Sonographer: Diana
Villagran, RDCS
Reading Physician:
Roger Muse MD
Stress Supervisor:
Ordering Physician: Roger Muse MD
AUC: Symptoms:chest pain, shortness of
breath, palpitations, TIA, stroke, peripheral
embolic

event (A)
Indication/Diagnosis: ACUTE ON CHRONIC SYSTOLIC CONGESTIVE
HEART FAILURE

FindingsPATIENT NAME: WALLACE,LARRY B ACCOUNT #: M267711402

Left Atrium:
The left atrium is moderately dilated. The left atrial volume is normal.
IAS:
The atrial septum appears normal.
Left Ventricle:
The left ventricular chamber size is mildly dilated. Left ventricular systolic function is at the lower limits of normal. The
EF range is estimated to be between 50 % and 55 % visually. No regional wall motion abnormality is observed. No left ventricular hypertrophy is observed. Right Atrium:
The right atrial cavity size is normal.
Right Ventricle:
The right ventricular cavity size is normal. The right ventricular global systolic function is normal. The pulmonary artery pressure is normal.
Mitral Valve:
The mitral valve is normal in appearance and function. There is no mitral valve leaflet thickening noted. There is mild to moderate mitral regurgitation observed. There is no evidence of mitral stenosis.

Diastolic Function
Normal diastolic LV function. Left atrial filling pressure is normal to low.
Aortic Valve:
The aortic valve appears tri-leaflet. There is no evidence of aortic valve thickening. Mild aortic leaflet calcification is

Patient: LARRY WALLACE MRN: X1821 Study Date: 06/09/2021
02:20 PM Page 1 of 3

visualized. There is no evidence or aortic regurgitation. There is no evidence of aortic stenosis.
Tricuspid Valve:
The tricuspid valve is normal in appearance and function. Mild tricuspid valve regurgitation is observed.
Pulmonic Valve:
The pulmonic valve is normal in appearance and function. Trivial pulmonary regurgitation.
Aorta:
The aorta appears normal without
significant plaque. Great Vessels:

IVC:The IVC is normal in size. The IVC is less than 21mm and collapses more than 50% with inspiration (RAP 1-5 mmHg).
Pericardium:
The pericardium is normal in appearance. No pericardial effusion is observed. No pleural effusion noted.

Exam DetailsProcedure Ordered: Transthoracic Echocardiogram 2D
Spectral Color

PATIENT NAME: B ACCOUNT #:

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Flow Doppler
Image Quality: Excellent
Exam Room: TC- Non Invasive Vascular (ECHO/NIVL)

Left Atrium Measurements
Label Value Normal Value
LA/AO Ratio, MM 1.86 (0.87 - 1.11)
LADs, 2D 47 mm (30mm - 40mm)
LADs, MM 50 mm (30mm - 40mm)
LAESV index, A-L 31.3 ml/m2 (16ml/m2 - 34ml/m2)
LAESV index, AL2 30.9 ml/m2 (16ml/m2 - 34ml/m2)
LAESV index, AL4 28.3 ml/m2 (16ml/m2 - 34ml/m2)

LAESV index, MOD2 29.6 ml/m2 (16ml/m2 - 34ml/m2)

Left Ventricle Measurements
Label Value Normal Value
LV Mass, MM ASE 244.47 g (88g - 224g)
LVEDV, MOD4 105 ml (62ml - 150ml)
LVEF, BP 52 % (52% - 72%)
LVEF, MM 56 % (52% - 72%)
LVOT PGmax 5 mmHg
LVOT PGmean 3 mmHg
LVOT Vmax 1.09 m/s (0.7m/s - 1.1m/s)
LVOT VTI 25.64 cm (18cm - 22cm)
LVPWd, MM 11 mm (6mm - 10mm)
MV E' lateral 0.06 m/s (0.1m/s - 999m/s)
MV E Vmax 1.23 m/s
MV E/E' lateral 21.03 (4.17 - 11.25)

Right Ventricle Measurements
Label Value Normal Value
TAPSE 15.7 mm (16mm - 20mm)

Mitral Valve Measurements
Label Value Normal Value
MR Defect Area (ERO) 0.06 cm2
MR PISA Alias V. 30.0 cm/s
MR PISA Radius 4 mm
MR Reg. Volume 9 mL
MV PGmax 9 mmHg

Patient: MRN: Study Date: 06/09/2021
02:20 PM Page 2 of 3

MV PGmean 2 mmHg (1mmHg - 2mmHg)
MV PHT 0.060 s
MV PHT 63 ms
MV VTI 50.37 cm
MVA D (continuity eq.) 1.9 cm2 (4cm2 - 6cm2)
MVA PHT 3.5 cm2 (4cm2 - 6cm2)

PATIENT NAME: B ACCOUNT #:

Aortic Valve Measurements
Label Value Normal Value
AV PGmax 17 mmHg
AV PGmean 9 mmHg
AV VTI 42.76 cm
AVA D (continuity eq. Vmax) 2.0 cm2
AVA D (continuity eq. VTI) 2.3 cm2
AVA Index (continuity eq. 0.99 cm2/m2 VTI)
AVA Index (continuity 0.86 cm2/m2
eq.Vmax)
LVOT Vmax / AV Vmax 0.53
LVOT VTI / AV VTI 0.60
LVOTd 22 mm (19mm - 21mm)

Pulmonic Valve Measurements Label Value
Normal Value

Great Vessels Measurements
Label Value Normal Value
IVC, 2D 16 mm (12mm - 23mm)

Electronically signed by Roger Muse MD on 06/09/2021 at 03:29 PM (No Signature Object)

Patient: MRN: Study Date: 06/09/2021
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		PATIENT NAME: ACCOUNT #: Electronically Signed by Roger K Muse, MD on 06/09/21 at 1531 PATIENT NAME: ACCOUNT #:

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CATH| CATHETERIZATION REPORT	202103-10 22:38	SAN ANTONIO, TEXAS 78201 PATIENT'S NAME: UNIT NO: DOB: AGE: 67 SEX: M ACCOUNT NO: ATTENDING PHYS: Dr. Shiv Kumar Goel, MD PT TYPE: DIS IN ROOM NO: J.241 DATE OF ADMISSION: 02/25/21 DATE OF DISCHARGE: 03/10/21 DATE OF PROCEDURE: 03/09/2021 PROCEDURE PERFORMED BY:Geetha Ramaswamy, MD REFERRING PHYSICIAN: Dr. Manuel Garcia. INDICATION FOR PROCEDURE: 1. Non-ST-elevation MI, abnormal stress test. 2. LV systolic dysfunction. PROCEDURE DONE:1. Selective coronary angiography. 2. PCI and stenting of the RCA. 3. Pressure wire assessment of the LAD. DESCRIPTION OF PROCEDURE:Informed consent was obtained from the patient after explaining risks, benefits, and alternatives in detail. Access was obtained in the right radial artery after modified Allen's test confirmed good dual blood supply. Subsequently, a 6-French hydrophilic sheath was placed in the right radial artery. A 6-French

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PCI to the RCA: After engaging the RCA with a 6-French JR4 guide catheter. IV heparin was used for anticoagulation. A Runthrough wire was advanced through the RCA and parked in the distal portion of the RCA. The RCA continued to exhibit a significant area of stenosis in the proximal and mid portion as described in the diagnostic catheterization report yesterday. There was an 80-90% proximal area of stenosis and the same degree of stenosis was seen in the mid portion of the RCA. I predilated the proximal and mid portion of the RCA with a 2.5x15mm compliant balloon using 2-3 separate inflations of up to 12 atmospheres following which IC nitroglycerin was given. There was still residual stenosis. There was TIMI 3 flow before PCI, of note. Still, there was significant residual stenosis post balloon angioplasty. Since there was significant residual stenosis after balloon angioplasty, I post-dilated the entire mid and proximal stenotic segments of the RCA. I placed a 3.0x24mm Synergy drug-eluting stent in the mid RCA and deployed this at 16 atmospheres. More proximally, I laid a 3.0x28mm Synergy drug-eluting stent in an overlapping fashion with the mid stent and deployed this at 16 atmospheres. I then post-dilated the entire stented segment of the RCA with a 3.5x20mm noncompliant balloon using high pressure inflations of 16-18 atmospheres. Final angiogram revealed excellent results with very well expanded stents in the proximal and CATHETERIZATION REPORT

PATIENT NAME: ACCOUNT #:

mid RCA and good TIMI 3 flow down the vessel.

Pressure wire assessment of the LAD: I then engaged the LAD with a 6-French XB 3.5 guide catheter with IV heparin on board. Advanced a pressure wire into the LAD after normalization in the proximal reference segment of the vessel. The wire was advanced into the distal LAD. IFR values of 0.92 were recorded on 2 separate occasions consistent with non-flow-limiting disease in the LAD.

SEDATION:The patient did receive moderate sedation as ordered by me, namely fentanyl 50mg and Versed 1mg. An independent trained observer, namely RN Amy Cameron, monitored vital signs throughout the procedure and I monitored his vital signs as well throughout the procedure.

IMPRESSION:1. Status post successful percutaneous coronary intervention to the right coronary artery with overlapping drug-eluting stents resulting in TIMI 3 flow.
2. Non-flow-limiting disease in the left anterior descending based on pressure wire assessment.
The patient had been loaded with Plavix in the cath lab yesterday. He was given an additional 300mg bolus today. Results were discussed with Dr. Garcia.

Geetha Ramaswamy, MD

IN: CATH/J.CCL/RAMGE
DD: 03/09/2021 0943
DT: 03/09/2021 1001
JOB ID: 5802230

Cc:Authenticated by Geetha Ramaswamy, MD On 03/10/2021 10:38:11 PM

cpcs rpt# 0309-0002

CATHETERIZATION REPORT

PATIENT NAME: ACCOUNT #:

Electronically Signed by Geetha Ramaswamy, MD on 03/10/21 at 2238

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		CATHETERIZATION REPORT PATIENT NAME: ACCOUNT #:

CASE| CASE MANGEMENT REPORT	202103-10 17:09	6700 IH-10 WEST SAN ANTONIO, TX 78201 PATIENT'S NAME: UNIT NO: DOB: AGE: 67 SEX: M ACCOUNT NO: ATTENDING PHYS: Dr. Shiv Kumar Goel, MD PT TYPE: DIS IN REPORT TYPE: CASE MANGEMENT REPORT ROOM NO: J.241 DATE OF ADMISSION: 02/25/21 DATE OF DISCHARGE: 03/10/21 --- HCM SUPPORT SERVICES --- HCM Support Services User Fields: Comments:HOME W/ HOME HEALTH: Carter Healthcare - Pearsall 101 S Oak Street Unit 101-B PEARSALL, TX 78061 Phone: (800) 259-2233 Fax: (888) 622-4329 Discharge Disposition: 06 - Home Health Score: 0.16434558 Level: Readmit Risk - Low Date/Time: 2/27/2021 6:07 AM --- Date Entered: 2/26/2021 Service Type: *Case Management Case Worker: Hill,Theresa Worklist Date: 3/10/2021 Service: Cardiac Rehabilitation Payer: HUMANA MCARE ERS PPO ADV Comments:--- 3/10/2021 02:42 PM by Theresa Hill --HOME W/ HOME HEALTH: Carter Healthcare - Pearsall 101 S Oak Street

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Unit 101-B
PEARSALL, TX 78061
Phone: (800) 259-2233
Fax: (888) 622-4329

PATIENT NAME: B ACCOUNT #:

--- 3/9/2021 04:34 PM by Theresa Hill ---
CM met with pt at the bedside to discuss DCP options and PT recommendations. Pt declines SNF or ARU placement, pending MD clearance to DC home; Establishing diuretic regimen/ optimize meds. Pt is agreeable to any home health that accepts his insurance and services Moore, TX region.

--- 3/9/2021 10:25 AM by Theresa Hill ---
3/8 OT rec's: ARU. CM awaiting updated PT recommendations for DCP.

--- 3/8/2021 03:41 PM by Theresa Hill ---
Cardiac Rehab: OP CR written materials with follow up instructions placed in pt's medical chart.

--- 3/5/2021 09:48 AM by Katie Saenz ---
CONSULT RECEIVED FOR LIFVEST. CM FAXED LIFEVEST ORDER AND CLINICAL TO ZOLL. CM NOTIFIED ZOLL OF LIFEVEST FAXED.

--- 3/4/2021 12:32 PM by Katie Saenz ---
PATIENT IS PENDING A NUCLEAR STRESS TEST TODAY.

RURAL PATIENT NAVIGATOR STATED SWING BEDS ARE AN OPTION IF PATIENT NEEDS PLACEMENT AT DC. PER RURAL PATIENT NAVIGATOR- PATIENT LIVES CLOSE TO FRIO REGIONAL SWING BED BUT HIS PCP CAN FOLLOW AT MEDINA REGIONAL SWING BED.

--- 3/1/2021 03:13 PM by Katie Saenz ---
PER PROGRESS NOTES-PATIENT ACUTELY CONFUSED 02/28; TRANSFERRED TO ICU LEVEL OF CARE.

--- 2/26/2021 07:04 PM by Veronica Morales ---
MET WITH PATIENT AT BEDSIDE. PATIENT IS 67 YEARS OLD, SCHEDULED FOR HEART CATH MONDAY 3/1/21. REPORTS LIVING WITH SPOUSE - DEBORAH WALLACE 830-267-0089 AT ADDRESS LISTED IN MOORE TEXAS. REPORTS PCP AS DR. ZACHARY WINDROW. REPORTS HE DOES NOT HAVE MPOA. REPORTS BEING INDEPENDENT AND WAS NOT RECEIVING ANY SUPPORT SERVICES PRIOR TO ADMISSION NOR USING ANY DME'S. REPORTS SPOUSE WILL TRANSPORT UPON DISCHARGE. REPORTS USING WALMART PHARMACY IN DEVINE TEXAS. NO NEEDS REPORTED AT THIS TIME. LMSW WILL CONTINUE TO FOLLOW UP.

Date Entered: 3/10/2021
Service Type: Home Health Agency
Case Worker: Morales,Veronica Worklist Date:
1/1/2006

Agency: Carter Healthcare - Pearsall
Referral Status:Booked

Date Entered: 3/10/2021
Service Type: DME Supplier
Case Worker: Morales,Veronica
Worklist Date: 1/1/2006

Agency: Zoll Lifecor Corp

PATIENT NAME: ACCOUNT #:

Referral Status:Booked

HCM DISCHARGE PLANNING

HCM Discharge Planning Comments:----------
HCM DISCHARGE PLANNING EVALUATION

Case Workers: Morales,Veronica
Living Status: Spouse-partner
Setting: Home-residence

ADL Limits: None or n/a

DME:None

HCM Discharge Planning User Fields:

Community Services Prior to Admission: None or NA Current Mental

Status/Cognition: Alert and Oriented

Information obtained from: Patient
Medical Records
Discharge Barriers, select all that apply: None or NA

Readmission (unplanned) in the last 90 days: No

Patient goals and preferences after discharge: PENDING PROGRESS - HEART CATH MONDAY 3/1/21

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Based on information gathered, is it likely that the patient's care needs can be met in the environment from which he/she entered the hospital?: Yes

Proposed Discharge Plan, select one: Home

Home Services needed: None or NA

If a caregiver is needed, is there a caregiver available, willing and capable to provide care?: Not Needed

Community services needed: None

New DME required at discharge: None or NA

If new DME is required, is patient able to obtain DME?: Not Applicable

Home Modifications required: None or NA

If home modifications are required, is patient able to obtain them?: Not Applicable

Patient concerns about obtaining medications on day of discharge?: None

PATIENT NAME: ACCOUNT #:

Transportation needs at discharge: NONE

Discharge Plan Discussed with:Patient

Have you discussed with the patient how his/her care needs may change over time?: Yes

Patient/representative agrees with discharge plan: Yes

Discussed expected insurance coverage and/or out of pocket expenses: No

Comments:: PLEASE SEE SS COMMENTS.

Date / Time: 2/26/2021 5:10 PM

Evaluated by: Morales,Veronica

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Updated by: INTERFACE,HIS (JXFPO8017) - 3/10/2021 5:08 PM

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PATIENT NAME: ACCOUNT #:

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EKG| ELECTROCARDIOGRAM REPORT	202103-09 08:13	6700 IH-10 WEST SAN ANTONIO, TEXAS 78201 PATIENT'S NAME: B UNIT NO: DOB: 09/28/53 AGE: 67 SEX: M ACCOUNT NO: ATTENDING PHYS: Dr. Shiv Kumar Goel, MD PT TYPE: ADM IN ROOM NO: J.241 DATE OF ADMISSION: 02/25/21 DATE OF DISCHARGE: Test Reason :Blood Pressure : ***/*** mmHG Vent. Rate : 067 BPM Atrial Rate : 067 BPM P-R Int : 194 ms QRS Dur : 078 ms QT Int : 412 ms P-R-T Axes : 085 -48 041 degrees QTc Int : 435 ms Sinus rhythm with marked sinus arrhythmia and PAC's Left axis deviation Low voltage QRS Abnormal ECG When compared with ECG of 08-MAR-2021 09:56, PR interval has decreased Confirmed by ESPINOZA, M.D., JUDE (4029) on 3/9/2021 8:12:58 AM Referred By: Shiv Goel Confirmed By:JUDE ESPINOZA, M.D. cpcs rpt# 0309-0017 Electronically Signed by Jude X Espinoza, MD on 03/09/21 at 0813 ELECTROCARDIOGRAM REPORT PATIENT NAME: ACCOUNT #:

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ICON| CONSULTATION REPORT	202103-08 16:12	6700 IH-10 WEST SAN ANTONIO, TX 78201 PATIENT'S NAME: UNIT NO: DOB: AGE: 67 SEX: M ACCOUNT NO: ATTENDING PHYS: Dr. Ravi Kanth R Yalamuri, MD PT TYPE: ADM IN REPORT TYPE: CONSULTATION REPORT ROOM NO: J.241 DATE OF ADMISSION: 02/25/21 DATE OF DISCHARGE: DATE OF CONSULTATION: 02/28/2021 CRITICAL CARE MEDICINE CONSULTATION REASON FOR CONSULTATION: Acute renal failure. CHIEF COMPLAINT:Shortness of breath. HISTORY OF PRESENT ILLNESS:This patient is a 67-year-old obese male with history of diabetes, hypertension, dyslipidemia, CHF, chronic back pain, transferred from Medina Regional Medical Center on February 25th with shortness of breath for 2 days with associated leg edema, chest tightness, mild nausea, and nonproductive cough and dizziness. He was found to have evidence of non-ST elevation MI and CHF exacerbation and was subsequently given IV diuretics and subsequently transferred to this facility for ongoing medical management. He was seen by both general cardiology and electrophysiology regarding cardiac ectopy. The patient underwent consultation by nephrology on the 26th due to development of acute renal failure; however, the past 24 hours, labs unfortunately revealed worsening renal insufficiency

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with hyperkalemia and renal dysfunction with a creatinine originally of 1.3 now 4.5. We have been asked by nephrology to place vascular access for initiation of hemodialysis. The patient is currently hemodynamically stable.

REVIEW OF SYSTEMS:Fourteen systems reviewed, pertinent positives as above, otherwise negative.

PAST MEDICAL HISTORY:Diabetes, hypertension, CHF, dyslipidemia, obesity, chronic back pain, sleep apnea.

SOCIAL HISTORY:No active tobacco, alcohol, or drugs.

ALLERGIES:CODEINE.

MEDICATIONS:See reconciliation.

PATIENT NAME: ACCOUNT #:

FAMILY HISTORY:Noncontributory.

PHYSICAL EXAMINATION:VITAL SIGNS:Temperature 36.5 Celsius, blood pressure is 104/68, respiratory rate 18, temperature 96% on room air, pulse 52.
GENERAL: The patient is alert, but somewhat sluggish, following commands.
HEENT: Pupils are reactive. Oropharynx is dry.
NECK: Supple. Trachea is not elevated.
CHEST: Diminished.
CARDIOVASCULAR: Regular.
ABDOMEN: Soft, nontender.
EXTREMITIES: Edema with digital ischemia, digital cyanosis. NEUROLOGIC: Moves all extremities.

DIAGNOSTIC DATA:CT head negative. Renal ultrasound shows no hydronephrosis. Chest x-ray shows cardiomegaly with vascular congestion.

LABORATORY DATA:Renal panel: Sodium 133, potassium 6.4, chloride 95, CO2 of 18, glucose 137, BUN 80, creatinine 4.5. White blood count 14, hematocrit 39.9, platelets 86.

IMPRESSION:1. Acute renal failure without evidence of obstruction based upon ultrasound. Differential certainly includes medication induced versus thromboembolic versus acute tubular necrosis.
2. Acute thrombocytopenia, rule out heparin-induced thrombocytopenia versus disseminated intravascular coagulation versus medication effect.
3. Exacerbation heart failure.

PLAN AND RECOMMENDATIONS:1. Check liver function test, ____, DIC panel, heparin antibody panel.
2. Place vascular access for hemodialysis.
3. Monitor closely in ICU.

David A Marks, MD

IN: ICON/J.MR/MARDAA
DD: 02/28/2021 1503
DT: 02/28/2021 1928
Job #: 5794617

Cc:Authenticated by David A Marks, MD On 03/08/2021 04:11:56 PM

cpcs rpt#: 0228-0022

PATIENT NAME: ACCOUNT #:

Electronically Signed by David A Marks, MD on 03/08/21 at 1612

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		PATIENT NAME: ACCOUNT #:

	202103-08 16:12	6700 IH-10 WEST SAN ANTONIO, TX 78201 PATIENT'S NAME: UNIT NO: DOB: AGE: 67 SEX: M ACCOUNT NO: ATTENDING PHYS: Dr. Ravi Kanth R Yalamuri, MD PT TYPE: ADM IN REPORT TYPE: OPERATIVE REPORT ROOM NO: J.241 DATE OF ADMISSION: 02/25/21 DATE OF DISCHARGE: DATE OF PROCEDURE: 02/28/2021 PROCEDURE PERFORMED:Ultrasound guided non-tunneled hemodialysis catheter placement. POSTOPERATIVE DIAGNOSIS, IMPRESSION, PLAN: PROCEDURE PERFORMED BY:David Marks, MD ASSISTANT:None unless otherwise specified. ANESTHESIA: (See anesthesia record.) SPECIMEN:None unless otherwise specified. ESTIMATED BLOOD LOSS: 5mL. INDICATION:Acute renal failure, hyperkalemia. DESCRIPTION OF PROCEDURE:Informed consent was obtained from the patient's family. Next, the patient in supine position. The right neck was then sterilely prepped with chlorhexidine solution and draped in usual sterile fashion. Now with ultrasound guidance, the right internal jugular vein was identified with appropriate anatomy. Next, approximately 3mL of 1% lidocaine was then infiltrated. Next, a single venotomy was made and a guidewire was passed without resistance. The track underwent serial dilation with successful placement of a triple-lumen Quinton catheter. The guidewire was then removed without resistance. All 3 ports aspirated and flushed easily. Catheter was secured to the patient via suture and a sterile dressing was applied. No immediate complications. Estimated blood loss 5mL. Chest x-ray currently pending. David A Marks, MD IN: IOP/J.MR/MARDAA DD: 02/28/2021 1642 PATIENT NAME: ACCOUNT #: DT: 02/28/2021 2050 Job #: 5794690 Cc:Authenticated by David A Marks, MD On 03/08/2021 04:11:57 PM cpcs rpt#: 0228-0032

Incessant Healthcare Private ltd

PATIENT NAME: ACCOUNT #:

Electronically Signed by David A Marks, MD on 03/08/21 at 1612

Incessant Healthcare Private ltd

		PATIENT NAME: ACCOUNT #:

	202103-05 07:17	6700 IH-10 WEST SAN ANTONIO, TEXAS 78201 PATIENT'S NAME: UNIT NO: DOB: AGE: 67 SEX: M ACCOUNT NO: ATTENDING PHYS: Dr. Ravi Kanth R Yalamuri, MD PT TYPE: ADM IN ROOM NO: J.206 DATE OF ADMISSION: 02/25/21 DATE OF DISCHARGE: Methodist Texsan Hospital 6700 Interstate 10 San Antonio, Texas 78201 210-736-6700 Name: MPI#: Facility MR#: Admission Number: Study Date: 03/04/2021 Study Time: 01:04 PM Date Of Birth: 09/28/1953 Age: 67 year(s) Height: 67 in. (170.2 cm) Weight: 250 lbs. (113.40 kg) BSA: 2.22 m2Gender: Male Rhythm: Normal sinus rhythm Heart Rate: 64 bpmEmergency: Infection: Contrast Allergy:NM Myocardial Prf Rst/Strs-Card Procedure StaffNurse: Iris Sloan, Nurse Nuclear Technologist: Vinnie Orduna Nuclear Technologist: Sebastian Ramirez, NMT Stress Supervisor: Manuel Garcia MD Ordering Physician: Manuel Garcia MD Reading Physician: Manuel Garcia MD Referring Physician: Goel,Shiv Kumar Procedure explained to patient, informed consent obtained AUC: Intermediate pre-test probability for CAD regardless of ECG of exercise ability - A Indications/Diagnosis: Ischemic Cardiomyopathy Anatomy: Male Rhythm: Normal sinus rhythm NUCLEAR CARDIOLOGY PATIENT NAME: WALLACE,LARRY B ACCOUNT #: J602091910 Room: Methodist Texsan Conclusions - The perfusion imaging study was abnormal. - stress portion of exam was supervised by myself. LVEF 33%mild-moderate inferior partially reversible defect. medical therapy. Clinical History and Risk Factors Family History of CAD: Yes Hypertension Yes Obesity: Yes Obstructive Sleep Apnea: Yes Isotope + Imaging Times Rest Isotope Admin Date / Time: 03/04/2021 10:01 Rest Imaging Time: 11:40 Rest Inject to Image Time: 99 min Heart Trust Clinic Patient: Printed: 03/05/2021 07:17 AM MRN: X1821 Page 1 of 3 Stress Isotope Admin Date / Time: 03/04/2021 12:15

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Stress Imaging Time: 12:40
Stress Isotope to
Image Time: 25
min Stress Agent
to Isotope
00:00:10
Administration
Time:

Exercise Time 01:00

Procedure:NM Myocardial Prf Rst/Strs-Card
Technique:A pharmacological rest-stress isotope administration protocol was utilized. The isotope was injected utilizing the IV line located in the left arm. The patient received IV administration of 10.9 mCi of 99mTc Tetrofosmin for purposes of rest perfusion imaging. Regadenoson was infused at a dose of 0.4 mg for 10 seconds. At 12:15 on 03042021 , the patient was given 29.9 mCi of 99mTc Tetrofosmin. Stress was continued for 4 minute(s) after stress isotope injection. Supine gated SPECT imaging of the myocardium was performed. Standard cardiac field of view was used for image acquisition. Computer assisted reconstruction was performed by standard gated SPECT techniques for the generation of reconstructed horizontal, vertical long-axis, and short axis projections. Polar Maps with semi-quantitative (quantitative) results were generated. Cedar's QGS software was utilized for processing. NUCLEAR CARDIOLOGY

PATIENT NAME: ACCOUNT #:

Pre Assessment:- Reason for Test
- Cardiomyopathy ECG Findings:
- Resting ECG - Normal Sinus Rhythm.
- Stress Arrhythmias - Ventricular escape beats.

Stress Findings:
- Reason for Termination - protocol completion.
- The resting heart rate was 64 bpm.
- The heart rate at peak stress was 71 bpm. The patient achieved 46 % of the maximal predicted heart rate. The 85% predicted maximal heart rate was 153 bpm.
- Heart rate Response - Attenuated secondary to pharmacologic agent adm - The resting blood pressure was 106/69 and increased to 110/71 , which is a normal BP response to pharmacologic administered (rest - exercise).
- MAX Load was 1.00 METS.
- Chest Pain - no chest pain.
- Functional capacity - could not be adequately assessed.

Raw Cine Images / Artifact / Incidental Findings
The rotating raw images were examined by the interpreting physician for motion and abnormal isotope uptake. Perfusion Images

The transient ischemic dilation (TID) ratio is 1.19. Wall Motion Findings:

The gated stress calculated left ventricular ejection fraction is 23%. The left ventricular stress EDV is 189 mL. The left ventricular stress ESV is 145 mL.

The gated rest calculated left ventricular ejection fraction is 33%. The left ventricular rest EDV is 162 mL. The left ventricular rest ESV is 108 mL. s

Pharmalogical Stress Examination Protocol
Stage Name Time in Load HR BP
Dosage ST Cardiac Cardiac Other Pain
Changes Symptom

Stage Level Arrhy Symp.
Supine 02:42 1.00 64 106/69 -0.050

mets bpm mm
Min 1 01:00 1.00 65 106/69 -0.150

mets bpm mm
04:26

1.00

69

109/68

-0.200

mets

bpm

mm

Heart Trust Clinic
Patient:
Printed: 07:17 AM
MRN: X1821

NUCLEAR CARDIOLOGY

PATIENT NAME: ACCOUNT #:

Page 2 of 3

Electronically signed by Manuel Garcia MD on
03/05/2021 at 07:16 AM (No Signature Object)

Heart Trust Clinic
Patient:
Printed: 03/05/2021 07:17 AM

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		MRN: X1821 Electronically Signed by Manuel M Garcia, MD on 03/05/21 at 0717 NUCLEAR CARDIOLOGY PATIENT NAME: ACCOUNT #:
- US Gallbladder	202103-01 17:58	

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EOP| OPERATIVE REPORT	202103-01 16:26	METHODIST TEXSAN HOSPITAL (COCMTA) OPERATIVE REPORT REPORT#:0301-0043 REPORT STATUS: Signed DATE:03/01/21 TIME: 1625 PATIENT: UNIT #: ACCOUNT#: ROOM/BED: J.206-01 DOB: AGE: 67 SEX: M ATTEND: Yalamuri,Ravi Kanth R MD ADM DT: 02/25/21 AUTHOR: Amalakuhan,Bravein MD RPT SRV DT: 02/28/21 RPT SRV TM: 1625 * ALL edits or amendments must be made on the electronic/computer document * OPERATIVE REPORT OPERATIVE NOTE NOTE: R.IJ Quinton/HD Catheter Placement: A time out was performed. My hands were washed immediately prior to the procedure. I wore a surgical cap, mask with protective eyewear, full gown and sterile gloves throughout the procedure. The patient was placed in Trendelenburg position. <Right chest region> was prepped using chlorhexidine scrub and draped in sterile fashion using a full drape and sterile probe cover employed. The medial and lateral heads of the sternocleidomastoid muscle were identified as was the carotid pulse. The <Internal Jugular vein> was identified using the ultrasound. Anesthesia was achieved over the vein using 1% lidocaine. Using real -time out of plane guidance, the introducer needle was inserted into the Internal Jugular vein under direct ultrasound visualization. Venous blood was withdrawn. The syringe was removed and a guidewire was advanced into the introducer needle. The guidewire was visualized in the Internal Jugular Vein by ultrasound. A small incision was made at the skin surface with a scalpel and the introducer needle was exchanged for a dilator over the guidewire. After appropriate dilation was obtained, the dilator was exchanged over the wire for a HD/Quinton catheter. The wire was removed and the catheter was sutured in place at <16 cm>. A sterile tegaderm shield was placed over the catheter at the insertion site. The patient tolerated the procedure without any hemodynamic compromise. At time of procedure completion, all ports aspirated and flushed properly. Post-procedure chest x-ray is pending at this time. Estimated blood loss is 5ml. Electronically Signed by Amalakuhan,Bravein MD on 03/01/21 at 1626 RPT #: 0301-0043

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- US Doppler Lower Ext Vein Bil	202103-01 13:58	
ECHO| ECHOCARDIOGRAM REPORT	202103-01 11:29	6700 IH-10 WEST SAN ANTONIO, TEXAS 78201 PATIENT'S NAME: UNIT NO: DOB: AGE: 67 SEX: M ACCOUNT NO: J602091910 ATTENDING PHYS: Dr. Ravi Kanth R Yalamuri, MD PT TYPE: ADM IN ROOM NO: J.206 DATE OF ADMISSION: 02/25/21 DATE OF DISCHARGE: Methodist Texsan Hospital 6700 Interstate 10 San Antonio, Texas 78201 210-736-6700 Transthoracic Echocardiogram 2D Spectral Color Flow Doppler, 3D WO POST PROC Name: WALLACE, LARRY MPI#: X1821 Facility MR#: J00062114 Admission Number: J602091910 Study Date: 03/01/2021 Study Time: 10:43 AM Date Of Birth: 09/28/1953 Age: 67 year(s) Height: 67 in. (170.2 cm) Weight: 268 lbs. (121.56 kg) BSA: 2.29 m2Gender: Male Blood Pressure: 130 mmHg / 74 mmHgHeart Rate:

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75
bpmRhythm:
Unknown
Infection:
Emergency:
Contrast
Allergy:

Procedure StaffSonographer: Sara L Sanchez, Echo Tech
Reading Physician: Manuel Garcia MD
Ordering Physician: GARMAM
AUC: Symptoms:chest pain,
shortness of breath,
palpitations, TIA, stroke,
peripheral embolic

event (A)
Indication/Diagnosis: chest pain
Diagnosis comments: INPATIENT ROOM 206

FindingsLeft Atrium:
The left atrium is moderately dilated. The left atrial volume is enlarged.
ECHOCARDIOGRAM REPORT

PATIENT NAME: ACCOUNT #:

IAS:The atrial septum is not well visualized.
Left Ventricle:
LVEF 30-35%. The left ventricular chamber size is moderately dilated. There is moderately decreased left ventricular systolic function.
Abnormal regional wall motion is observed.
Borderline concentric ventricular hypertrophy is observed.
Global longitudinal strain (GLS) is abnormal (greater than -16%).
Right Atrium:
The right atrium is moderately dilated.
Right Ventricle:
Right ventricle is poorly visualized.
The right ventricular cavity size is normal. The pulmonary artery pressure is mildly increased.
Mitral Valve:
There is mild thickening of the mitral valve leaflets. There is moderate mitral regurgitation observed.
Aortic Valve:
The aortic valve leaflets are mildly thickened. Mild aortic leaflet calcification is visualized. There is no evidence or aortic regurgitation.
There is mild aortic sclerosis present.
Tricuspid Valve:
Moderate tricuspid valve regurgitation is observed.

Patient: LARRY WALLACE MRN: X1821 Study Date: 03/01/2021
10:43 AM Page 1 of 3

Pulmonic Valve:
Mild pulmonary regurgitation.
Aorta:
The aorta is not well visualized.
Pericardium:
The pericardium is not well visualized.

Exam DetailsProcedure Ordered: Transthoracic Echocardiogram 2D Spectral Color Flow Doppler, 3D WO POST PROC
Procedure Status: STAT study
Image Quality: Good
Exam Room: Methodist Texsan

Left Atrium Measurements
Label Value Normal Value
LADs, 2D 46 mm (30mm - 40mm)
LAESV index, A-L 40.2 ml/m2 (16ml/m2 - 34ml/m2)
LAESV index, AL2 33.2 ml/m2 (16ml/m2 - 34ml/m2)
LAESV index, AL4 44.5 ml/m2 (16ml/m2 - 34ml/m2)
LAESV index, BP 37.1 ml/m2 (16ml/m2 - 34ml/m2)
LAESV index, MOD2 31.9 ml/m2 (16ml/m2 - 34ml/m2)
LAESV index, MOD4 40.6 ml/m2 (16ml/m2 - 34ml/m2)

ECHOCARDIOGRAM REPORT

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PATIENT NAME: ACCOUNT #:

Left Ventricle Measurements
Label Value Normal Value
GLS, 2D, 3P -11.1 % (-99% - -18%)
LVEF, 3D 44 % (52% - 72%)
LVEF, BP 43 % (52% - 72%)
LVESV, MOD4 112 ml (21ml - 61ml)
LVOT PGmax 2 mmHg
LVOT PGmean 1 mmHg
LVOT
Vmax
0.78
m/s
(0.7m/s
-
1.1m/s)
LVOT
VTI
14.86
cm
(18cm -
22cm)
MV E' lateral 0.12 m/s (0.1m/s - 999m/s)
MV E' septal 0.10 m/s (0.07m/s - 999m/s)
MV E Vmax 1.18 m/s
MV E/E' lateral 9.94 (4.17 - 11.25)
MV E/E' septal 12.07 (5.45 - 13.33)

Right Ventricle Measurements
Label Value Normal Value
Diast. PAP (PR P diast. + 25 mmHg
CVP)

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Mitral Valve Measurements
Label Value Normal Value
MV PGmax 6 mmHg

MV PHT 0.080 s
MV PHT 78 ms
MV VTI 41.36 cm
MVA D (continuity eq.) 1.5 cm2 (4cm2 - 6cm2)
MVA PHT 2.8 cm2 (4cm2 - 6cm2)

Aortic Valve Measurements

Patient: MRN: X1821 Study Date: 03/01/2021
10:43 AM Page 2 of 3

Label Value Normal Value
AV PGmax 11 mmHg
AV PGmean 6 mmHg
AV VTI 28.01 cm
AVA D (continuity eq. Vmax) 1.9 cm2
AVA D (continuity eq. VTI) 2.2 cm2
AVA Index (continuity eq. 0.96 cm2/m2 VTI)
AVA Index (continuity 0.83 cm2/m2
eq.Vmax)
LVOT Vmax / AV Vmax 0.46
LVOT VTI / AV VTI 0.53
LVOTd 23 mm (19mm - 21mm)

Tricuspid Valve Measurements
ECHOCARDIOGRAM REPORT

PATIENT NAME: WALLACE,LARRY B ACCOUNT #: J602091910

Label Value Normal Value
RA Pressure 15 mmHg (0mmHg - 5mmHg)
RVSP 35 mmHg (25mmHg - 35mmHg)

Pulmonic Valve Measurements Label Value
Normal Value

Great Vessels Measurements
Label Value Normal Value
IVC, 2D 27 mm (12mm - 23mm)

Electronically signed by Manuel Garcia MD on 03/01/2021 at 11:27 AM (No Signature
Object)

Patient: LARRY WALLACE MRN: X1821 Study Date: 03/01/2021
10:43 AM Page 3 of 3

Electronically Signed by Manuel M Garcia, MD on 03/01/21 at 1128

ECHOCARDIOGRAM REPORT

PATIENT NAME: ACCOUNT #:

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ECON| Consultation Note	202103-01 09:56	METHODIST TEXSAN HOSPITAL (COCMTA) Consultation Note REPORT#:0228-0045 REPORT STATUS: Signed DATE:02/28/21 TIME: 1636 PATIENT: UNIT #: ACCOUNT#: ROOM/BED: J.206-01 DOB: AGE: 67 SEX: M ATTEND: Yalamuri,Ravi Kanth R MD ADM DT: 02/25/21 AUTHOR: Couch,Linda S MD RPT SRV DT: 02/28/21 RPT SRV TM: 1636 * ALL edits or amendments must be made on the electronic/computer document * CONSULT NOTE NOTE: hem onc consult note 239063 67 yo male, recent nstemi worsening thrombocytopenia over last few days. concern for ttp. platelts have dropped from 190 to 86, sharpest deline in last 48 hrs (however still 86K on last lab) pts creatinine has also increased from 1.3 to 4.5, again c greastest change in last 48 hrs inr slightly elvated, reck pending hemolysis--ck haptoglobin, indirect bili, direct coombs, fibrinogen, d-dimer, ck ADAMTS 13 activity cbc q 6 x4 inr q 6 x4 currently on the PLASMIC scale pt scoring low, only pt due to change in creatinine. however, given overall clinical picture agree c close monitroing Diagnosis of TTP cannot be made without the presence of schistocytes. Pathological smear review requested. Electronically Signed by Couch,Linda S MD on 03/01/21 at 0956 RPT #: 0228-0045
- Chest 1 View	202102-28 16:58	
- CT Head	202102-28 09:29	
- US Renal Evaluation	202102-26 16:52	
- Chest 1 View Portable	202102-26 10:25	

Procedures

Name	Dates	Details	Sending Facility

S AND ADVERSE REACTI

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Name	Dates	Details	Sending Facility
codeine	2021-02-25 15:40	Type: CODEINE; Value: F006001550; Descr: codeine	Methodist Texsan Hospital

Results

HAPTOGLOBIN

Name	Dates	Additional Notes	Details	Facility
Name: HAPT Descr: HAPTOGLOBIN	2021-02-28 16:31	132 mg/dl	Normal Range:31-200; Normality: N; Result Status: F	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital

HEPATITIS B SURFACE AG

Name	Dates	Additional Notes	Details	Facility
Name: HBSAG Descr: HEPATITIS_B_SURFACE_AG	2021-03-01 13:51	NON REACTIVE	Normal Range:NONREACTIVE; Normality: ; Result Status: F	Metropolitan Methodist Hospital
Name: UNK Descr: UNK	2021-03-01 13:51		Normal Range:; Normality: ; Result Status:	Metropolitan Methodist Hospital

PF4 HEP-DEP PLT AB IGG

Name	Dates	Additional Notes	Details	Facility
Name: PLTABST Descr: PF4_HEP-DEP_PLT_AB_IGG	2021-02-28 16:31	NEGATIVE	Normal Range:; Normality: ; Result Status: F	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital
Name: PF4OD Descr: PF4_INITIAL_OD	2021-02-28 16:31	0.004 units	Normal Range:; Normality: ; Result Status: F	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital

HEPATITIS B SURFACE AG

Name	Dates	Additional Notes	Details	Facility
Name: HBSAG Descr: HEPATITIS_B_SURFACE_AG	2021-02-28 16:31	NON REACTIVE	Normal Range:NONREACTIVE; Normality: ; Result Status: F	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital
Name: HBSABQNT Descr: HEPATITIS_B_SURFACE_AB_QUANT	2021-02-28 16:31	<5.00 miu/ml	Normal Range:<5.00; Normality: N; Result Status: F	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital
Name: HBCAB Descr: HEPATITIS_B_CORE_AB	2021-02-28 16:31	NON REACTIVE	Normal Range:NONREACTIVE; Normality: ; Result Status: F	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital
Name: HBSAG Descr: HEPATITIS_B_SURFACE_AG	2021-02-28 16:31		Normal Range:NONREACTIVE; Normality: ; Result Status: I	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital
Name: HBSABQNT Descr: HEPATITIS_B_SURFACE_AB_QUANT	2021-02-28 16:31	<5.00 miu/ml	Normal Range:<5.00; Normality: N; Result Status: F	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital
Name: HBCAB Descr: HEPATITIS_B_CORE_AB	2021-02-28 16:31	NON REACTIVE	Normal Range:NONREACTIVE; Normality: ; Result Status: F	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital

PTH INTACT

Name	Dates	Additional Notes	Details	Facility

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Name: PARAIT Descr: PARATHYROID_HORMONE_INTACT	2021-02-27 07:43	179.70 pg/ml	Normal Range:14.00-72.00; Normality: H; Result Status: F	Metropolitan Methodist Hospital
Name: UNK Descr: UNK	2021-02-27 07:43		Normal Range:; Normality: ; Result Status:	Metropolitan Methodist Hospital

COMPLEMENT C3

Name	Dates	Additional Notes	Details	Facility
Name: COMC3 Descr: COMPLEMENT C3	2021-02-28 16:31	49 mg/dl	Normal Range:90-180; Normality: L; Result Status: F	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital
Name: COMC4 Descr: COMPLEMENT C4	2021-02-28 16:31	<6 mg/dl	Normal Range:10-40; Normality: L; Result Status: F	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital

RETICULOCYTE COUNT

Name	Dates	Additional Notes	Details	Facility
Name: RETICT Descr: RETICULOCYTE COUNT	2021-02-28 16:31	2.7 %	Normal Range:0.6-2.5; Normality: H; Result Status: F	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital
Name: RETIABS Descr: RETIC_ABSOLUTE_	2021-02-28 16:31	113 bil/l	Normal Range:15-90; Normality: H; Result Status: F	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital
Name: IRFRET Descr: IMMATURE_RETIC_FRACTION	2021-02-28 16:31	34.8 %	Normal Range:2-18; Normality: H; Result Status: F	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital
Name: RETHE Descr: RETIC_HEMOGLOBIN_CONTENT	2021-02-28 16:31	32.5 pg	Normal Range:28-36; Normality: N; Result Status: F	Methodist Hospital
Name: UNK Descr: UNK	2021-02-28 16:31		Normal Range:; Normality: ; Result Status:	Methodist Hospital

Encounters

Encounter	Location	Date
OUTPATIENT	Methodist Texsan Hospital	2021-02-25 15:42
INPATIENT	Methodist Texsan Hospital	2021-02-25 16:53
INPATIENT	Methodist Texsan Hospital	2021-02-26 16:51
OUTPATIENT	Metropolitan Methodist Hospital	2021-02-27 07:43
OUTPATIENT	Methodist Hospital	2021-02-28 16:31
INPATIENT	Methodist Texsan Hospital	2021-02-28 16:58
OUTPATIENT	Metropolitan Methodist Hospital	2021-03-01 13:51
INPATIENT	Methodist Texsan Hospital	2021-03-01 17:57
OUTPATIENT	Metropolitan Methodist Hospital	2021-06-09 14:00
OUTPATIENT	SES-OPTUM	2022-05-19 19:39

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